

“Jesus Speaks Better”: Interactions Between State and Faith-Based Organizations in Service Implementation

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Abstract

This article investigates the factors that enable cooperation between the state and faith-based organizations (FBOs) in public service implementation. Looking at the Brazilian context, the research explores the consequences of hybridization processes for FBOs and state organizations, as well as for public services. The article analyzes the case of therapeutic communities, FBOs that provide services for drug and alcohol users. Data were collected through documents and 67 interviews with different actors involved in the service implementation. The findings contribute to the literatures on FBOs, hybridization, and institutional fit, showing that the interactions between state organizations and FBOs lead to the inclusion of values and cognitive frameworks for both, generating a mutual constitution between them. The findings show how different types of institutional fit enable cooperation throughout different forms of mediation, which also affect the organizations involved in the cooperation.

Keywords

faith-based organizations, service implementation, hybridization, therapeutic communities

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Introduction

Religious groups provide diverse public services around the world (Bielefeld & Cleveland, 2013a, 2013b; Fu et al., 2021; Jeavons, 2004; Kearns et al., 2005; Sider & Unruh, 2004; Stritt, 2008; Wilson, 2003; Wuthnow, 2004/2009). This phenomenon has grown since the managerial reforms of the 1990s, when faith-based organizations (FBOs) became more heavily involved in the provision of welfare services (Bielefeld & Cleveland, 2013a). FBOs are organizations that have religion as part of their identity. They may promote different types of services, including promoting religious practices (Scheitle & McCarthy, 2018), alongside other services such as education, health, and others; however, they are always guided by religious principles/values (Markofski et al., 2020).

The participation of FBOs in service provision is part of a broader movement to hybridize policy arrangements (Beaton et al., 2021), in which different sectors—governmental, nonprofit, and private—cooperate in the implementation of public services (Bielefeld & Cleveland, 2013b; Considine & Lewis, 1999, 2003; Suykens et al., 2020; Wilson, 2003). Hybridization is conceived as a formal or informal arrangement through which different sectors and governance mechanisms overlap (Seibel, 2015). Its functioning depends on the ability of organizations to adapt to multiple organizational logics (Beaton et al., 2021; Fossetøl et al., 2015). As a result, hybridization produces diverse effects on public services and on the actors who implement them (Thomann et al., 2018).

A central theme in the literature about hybrid organizations is understanding how cooperation occurs and how organizations develop skills to move between different institutional environments in the production of policies (Kinney, 2006). Although there is an increased understanding of these processes (see, for example, Fu et al., 2021; Suykens et al., 2020), there are still gaps in understanding how hybridization affects the organizations involved, especially when very different institutional values are at stake (Beaton et al., 2021).

This is the case for FBOs, for example, where conflicting values may emerge with the state organizations with which they are supposed to cooperate (Göçmen, 2013; Kinney, 2006; Wilson, 2003). Moreover, this literature was mainly developed by analyzing the case of the United States, where FBOs have specific characteristics in their relationship with the state. How this cooperation happens, what enables it, and how the cooperation affects both FBOs and state organizations involved in this process are as yet unexplored questions, especially in contexts outside the United States. These are the questions that drive this article to explore what enables the cooperation between FBOs and state organizations in public service implementation in the Brazilian context, and the consequences of this cooperation for FBOs, state organizations, and services.

To answer these questions, an analytical framework of state–society interaction (Abers et al., 2018; Lavallo et al., 2012) and institutional fit (Skocpol, 1992) was adopted. According to this approach, the relationship between state and nonstate organizations produces a process of mutual constitution between these actors (Abers et al.,

2018; Lavalley et al., 2012). This relationship may occur through different types of institutional fit (Lavalley & Szwako, 2022). The hypothesis raised in this article is that when the state adopts categories introduced by nonstate actors, it partially modifies cognitive frameworks that shape its modes of organization and intervention, and it starts to see like these actors. The same is true for nonstate actors who, when cooperating with the state, also end up incorporating the values of state organizations (Lavalley & Szwako, 2022).

This article analyzes how connections emerge between the state organizations and FBOs contracted to provide public services, observing tensions, conflicts, and fits between and among them. To this end, the research covered the case of therapeutic communities (TCs) contracted by the Government of São Paulo to provide care services to problematic users of alcohol and drugs. The article explores the FBO–state organization interplays based on data collected through 67 interviews with FBO representatives and policy managers in government organizations.

The findings suggest that the cooperation between the state organizations and the FBOs is based on different forms of institutional fit that have consequences for both organizations, as they generate changes and require adjustments. This process is based on different types of mediation—both symbolic and positional. The capacity to establish mediation is important to explain cooperation, but, at the same time, to explain potential new conflicts. Accordingly, this article makes contributions to the literature about FBOs, hybridization, and institutional fit.

Besides this introduction, the article is divided into five sections. First, the theoretical debate is presented. Then, the context of TCs in Brazil and the methodology used are explained. The third section presents the findings, and the fourth presents a discussion making a dialogue between the findings and the literature. The article ends with final remarks.

State–Society Interactions in Service Implementation: The Case of FBOs

The new public management (NPM) reforms incorporated types of nonstate organizations into public service provision (Thomann et al., 2018). Via various partnerships or contracting arrangements, nonprofit and private organizations developed hybrid models of service provision with state organizations (Beaton et al., 2021). This hybridization process had several consequences for both service and provider organizations, as it requires the development of skills that allow organizations to move between multiple organizational logics (Fossetøl et al., 2015).

FBOs are among the organizations subcontracted to provide services. Although FBOs have been developing social services for centuries, this process became institutionalized through the NPM reforms, and FBOs became central to the delivery of some services, particularly welfare services (Bielefeld & Cleveland, 2013a, 2013b; Stritt, 2008). In this article, FBOs are organizations that are directly affiliated with religious congregations and/or denominations, develop religious activities, or both (Bielefeld & Cleveland, 2013a).

Studies in the U.S. context have analyzed the effects of FBOs on service delivery. They argue that, on one hand, FBOs can be more effective in some kinds of services, especially by ensuring their continuity and introducing specific agendas into social policies (Adloff, 2006; Bishop, 2006; Coleman, 2001; Fossett, 2004; Mapes, 2004). However, other scholars have questioned the benefits of FBO involvement for welfare policies, as they may have limited capacity and may compromise principles of the welfare state, such as inclusion and universalism (Dunn, 2009; Kinney, 2006; Poole, 2003; Poole et al., 2002).

Scholars have also analyzed the extent to which relationships with the state organizations change FBOs, as they assume various risks when receiving government resources (Wilson, 2003). Governments can influence the process of hiring employees, which can have an impact on the values of people who work in organizations (McClellan, 2004; Rosenthal, 2006). State organizations can also interfere in FBOs by conducting surveys that criticize the way services are provided (Lugo et al., 2008, 2009; VanSlyke et al., 2005). Finally, the government can influence the logistics and management of FBOs (Cnaan & Newman, 2010; Kinney, 2006; Sager, 2007) or even the scope and range of activities they carry out (Goode, 2006).

The literature also points out that relationships with the state organizations can produce several risks for FBOs. On one hand, the provision of public services can generate a process of depersonalization of service and organizational drift that increases procedures and formalities for FBOs (Bielefeld & Cleveland, 2013a; Ebaugh et al., 2007; Fu & Cooper, 2020). On the other hand, state organizations can help to promote institutional capacity in FBOs, especially in younger organizations (Bielefeld & Cleveland, 2013a; Fu et al., 2021).

Furthermore, when dealing with the state organizations, FBOs face the risk of their religious identity being undermined. To keep services running, organizations often need to have some degree of flexibility (Berger, 2003; Emerson & Hartman, 2006). In this context, FBOs see flexibility as a feature that helps survival, but it can also result in a retreat from their religious identity (Bielefeld & Cleveland, 2013a). One way to maintain religious identity is to include religion in routine treatment activities, which must be approved by the state organizations (Tangenberg, 2004).

Although the literature has advanced the understanding of how the relationship between FBOs and the state organizations affects the development of public services and changes them, it is not known how these relationships play out on an everyday basis (Bielefeld & Cleveland, 2013a; Freeman & Houston, 2010) and to what extent they transform FBOs, state organizations, and the services they provide. Moreover, the existing literature was mostly developed in the U.S. context, where FBOs and state organizations have specific characteristics and demands. These scholars found, for example, that FBOs tend to collaborate less with state organizations than other types of organizations due to the operational capacity and degree of religiosity (Fu et al., 2021). How much of this is true in other contexts where both FBOs have different trajectories and where state organizations depend on this cooperation is as yet unclear.

To answer these questions, this article breaks with the common dichotomous understanding of state and social actors, and instead starts from the idea that civil society

and the State are mutually constituted. The mutual constitution approach starts from a dynamic, procedural, and relational perspective on understanding the forms of interaction between the State and civil society (Lavalle & Szwako, 2022), in which the boundaries between these spheres are tenuously analyzed. This approach privileges the study of relational patterns, organizational dynamics, and the different interactional logics that permeate spaces for dialogue and construction between civil society and state organizations. It thus explores the forms of interaction between both spheres (Lavalle et al., 2019).

This view offers a helpful analytical approach to understand what is institutionalized as a result of the State–society interaction. By emphasizing mutual constitution, this approach acknowledges that social actors not only pressure the state externally, but also are incorporated into and gain institutional expression within, through, and against the State (Lavalle et al., 2019). We consider, in this article, that state–society interactions allow social actors to adapt to the institutional context and to construct their institutional fit with the State (Skocpol, 1992). The concept of “institutional fit” is defined as an institutional sedimentation of historical processes of socio–state interactions through which social actors use political institutions in their favor, thus expanding their capacity to act (Lavalle et al., 2019). Following Skocpol’s work, four types of institutional fit have been mapped by literature. The first is the symbolic fit, by which social actors articulate their demands and make them public using symbolic categories (Szwako & Lavalle, 2022). The second is positional fit, which identifies official positions as instances to be occupied by social actors (Abers & de Oliveira, 2015). The third is programmatic fit, referring to the institutionalization of projects and programs developed by social actors into public policies (Albuquerque, 2015). Finally, the fourth is technical–practical fit, which concerns policy instruments institutionalized through the relationship between the State and social actors (Marques, 2013). This article analyzes the institutional fit in FBO–state organization relationships, focusing on the symbolic and positional fit. We argue that, as governmental norms and guidelines constrain the fit, it is constructed in a dynamic process of disputes and negotiations on both sides. As a result, both the organizations and the services are affected.

Context and Method

FBOs in the Brazilian Context: The Case of TCs

To answer the question of how the cooperation between FBOs and the state organizations happens, what enables it, and how the cooperation affects the organizations in contexts outside the United States, this article presents an analysis of FBOs that provide public services for alcohol and drug users in Brazil. The choice of this case is justified for several reasons. Due to the NPM reforms in the 1990s, Brazil saw significant growth in the participation of religious organizations in service provision. In this period, the growing partnerships between FBOs and Brazilian state organizations involved formalizing activities already developed by these FBOs in places where the state was not present (Burity, 2007).

In Brazil, social work services are permeated by religion, as the history of religious organizations is intertwined with the country's history of social policy development (Behring & Boschetti, 2006; Quiroga, 2001). Religious philanthropic entities acted through charitable and welfare interventions. The 1988 Constitution recognized social assistance services as a right that should be guaranteed by the state for people with vulnerabilities (Sposati, 2007). Considering that the State should be laic, the constitution tried to diminish the space of religion in service provision, treating services as universal rights for all people in need. However, studies show that this religious detachment has not been applied to some services, such as the treatment of alcohol and drug use (Pires, 2018) in TCs.

TCs are nonprofit organizations regulated by the State. Most of them are established with the support of religious organizations. Their objective is to house and offer on-site treatment for people with disorders resulting from the problematic use of psychoactive substances (Pires, 2018). Some 95.6% of the Brazilian TCs use religious practice as a therapeutic methodology, even in organizations that are not connected to religious institutions or denominations. To most TCs, religiosity has a meaning based on Christian rituals such as Bible-reading sessions, participation in services, worship, and prayers (Institute of Applied Economic Research [IPEA], 2017). Moreover, religion is connected to the historical development of TCs. Brazilian TCs were influenced by the Oxford, Minnesota, and Synanon movements, which advocated providing treatment to alcoholics and problem users of drugs. This approach was anchored in Christian religious values, through a community model grounded in coexistence between peers and work therapy (Damas, 2013).

Religious practices in TCs have two objectives: to establish a standard of moral Christian conduct for users, and emotional and psychological support for maintaining abstinence (dos Santos, 2018). Thus, religiosity is seen as essential for overcoming addiction (Souza et al., 2018). In the methodology adopted by the TCs, being part of a religion increases the chances of social reintegration, because religious sociability tends to promote the reconstruction of the individual's habitus and to integrate the individual into a social protection network (Dutra & Arenari, 2017).

In recent years, TCs have increasingly tried to influence policy design, especially to be contracted by the state as organizations that can provide public services. One of the mechanisms adopted by TCs in this effort was to organize themselves into entities, such as associations and federations (IPEA, 2017) that mediate the relations between TCs and state organizations. These federations also certify TCs to determine which ones are official and can cooperate with the state (Pires, 2018).

During this process of increasing the cooperation between the TCs and state organizations, religious activities became a source of great conflict. On one hand, religious practices are central to the TCs' methodology. On the other hand, human rights and health organizations are opposed to public funding for religious activities by the State. This dispute entered the National Congress, where some deputies negotiated to approve laws that preserved the role of religiosity as a form of treatment offered by the TCs and funded by the state (Doudement & Da Conceição, 2018). The solution for this dispute was the inclusion of the term *spirituality* in the rules and guidelines as part of

the activities developed by the TCs, substituting for the idea of religiosity. In this article, our contribution is not to the discussion of the definition of spirituality or its differentiation from religion, but to the debate about how this category is mobilized by the actor and institutionalized in public service, and its effect on FBOs, the state, and the service provided.

FBOs Contracted by State Organizations: Programa Recomeço

This article presents an analysis of the TCs of *Programa Recomeço*. The *Programa Recomeço (Restart Program): A Life Without Drugs* is a program established by the state government of São Paulo in 2013 that carries out prevention, treatment, rehabilitation, reception, and social reintegration activities with those who use psychoactive substances problematically. *Recomeço* contracts and funds services from TCs, paying the individual expenses of service users. It offers treatment for 1,335 patients in 64 TCs across the State of São Paulo. TCs provide services based on coexistence between peers, the strengthening of family and community bonds, and the development of strategies for maintaining abstinence.

TCs contracted by the *Recomeço* receive referrals following prior evaluation from public health clinics that specialize in alcohol and drugs. Health clinics are managed by state organizations and are the gateway to TCs. The TCs analyzed here are contracted by the Drug Policy Coordination (COED) of the Secretariat for Social Development (SEDS) of the State of São Paulo. TCs adhere to a public selection processes through the Brazilian Federation of Therapeutic Communities (FEBRAC).

FEBRAC is a federation (association of TCs) that represents 21.4% of Brazilian TCs, and it is one of the oldest organizations in the sector (IPEA, 2017). Its institutional objective is to strengthen, organize, and provide training and advice to the TCs (FEBRAC, 2019). Historically, it was one of the main federations to offer professional training for professionals working in the field of alcohol and other drugs in Brazil. FEBRAC presents itself as the federation with the greatest alignment with state organizations (Barroso, 2021). However, in this relationship, FEBRAC also brings its own demands, such as the defense of the traditional practices of the TCs, which are responsible for the success of the work done in these spaces.

In the State of São Paulo, the role played by FEBRAC has been growing since 2013, and today it is the federation that mediates the relations between the TCs in the *Recomeço* Program and COED. FEBRAC is in charge of managing and monitoring vacancies distributed throughout the state, supervising and seeking to achieve uniformity in the execution of services, in addition to training the TCs workers. It also has the role of evaluating the impact of the activities developed and monitoring the TCs facilities (FEBRAC, 2019). All the TCs in this study are affiliated with FEBRAC.

Before they can be contracted by the State of São Paulo, TCs must meet certain targets: 12 mandatory inspections per year in each TC, carried out by FEBRAC, four training and professional development sessions over 12 months, and 70% of staff

having higher and 100% secondary education qualifications. The last prerequisite is related to the qualification of mid-level professionals, who generally include monitors and/or social educators (FEBRAC, 2019).

Data Collection

This article drew on two sources of data: documents and interviews. The first source was documents and regulations about the TCs in Brazil and about the *Recomeço* Program. Different types of documents (laws, decrees, ordinances, technical notes, resolutions) that aim to regulate the work of TCs in Brazil from 2001 (the year of publication of the first document that aimed to regulate the activities of TCs at the national level) to 2022 were selected and analyzed. In addition, documents referring to the *Recomeço* Program in São Paulo, from 2013 (the date of the program's creation) to 2022, were also analyzed. A list of documents analyzed within the scope of this article is in the appendix.

The second source was interviews with different actors involved in the case between 2019 and 2021. Three types of actors were interviewed: TC workers, representatives of the FEBRAC, and state organization managers. The purpose of this variation was to understand different perspectives about the relationship between the TCs and state organizations, and also how each type of actor adapts its actions based on these interactions.

First, 37 TCs were selected to provide diversity in the field: 12 of them self-identified as Evangelical organizations, 12 as Catholic, and 13 as ecumenical. Ecumenical organizations are those with no formal connections with any specific religion but that develop spiritual activities. In these TCs, interviews were conducted with 58 workers (32 psychologists and 26 social workers). Given the research question, there were no comparisons between TCs with different religions or between different types of professionals, which can be done in future work. These interviews were conducted in person, using a semi-structured guide. The guide addressed the practices of the TCs, how they organized their services, and the interactions between TCs and state organizations. Furthermore, two TCs were visited, and their directors were also interviewed.

Second, two COED managers were interviewed. Considering that the size of the team is small (five workers), the interviews were conducted with the two high-level managers of the program that belongs to this state organization. These interviews were done online using a semi-structured guide that addressed questions about the role of religion in the program, the COED's attempts to regulate the use of religion as a form of treatment, and conflicts arising from this interaction.

Third, three FEBRAC representatives were interviewed, all responsible for implementing and supervising the *Recomeço* Program. These interviews were also done online, using a semi-structured guide that addressed questions about the TCs' religious ties, their perspectives about the role of religion in the service, the relationship with the state organization and its potential conflicts, and, finally, the role of FEBRAC as a mediator in this relationship.

Data Analysis

For this article, 29 documents and 67 interviews were analyzed. All interviews were transcribed, and the materials were analyzed using NVivo. The coding process was developed through a thematic analysis following Braun et al.'s (2022) steps. The first step was familiarization, during which the authors read all the transcribed interviews and documents trying to make sense of the data based on the research question “how does the cooperation between FBOs and state organizations happen? What enables it and how does the cooperation affect organizations in contexts outside the United States?” After reading the material, the authors discussed their perceptions and ended up with two macro themes to guide the coding process: *spirituality/religiosity* and *interactions between state organizations and TCs*. The authors divided the interviews and coded the material again to fit the excerpts to these macro codes.

In the second step, based on new conversations, the authors generated new sub-codes for the macro codes. In this step, one of the authors read 10 interviews and generated codes inductively. Three codes emerged from this phase: *strategies to interact*, *changes in the organizations*, and *conflicts*. These codes were described and applied to the other interviews, which were coded by one author each. Finally, each interview was re-coded by another author to assure coherence in the coding process.

In the last step, the authors read the coded material and searched for relationships between the codes that could organize the narrative. Based on that, the findings presented in the next section are organized considering (a) how religiosity/spirituality was incorporated into policy, (b) the strategies adopted by state organization to regulate religiosity/spirituality, and (c) conflicts between the actors. To present the findings, we chose interviewees' excerpts that were representative of each code, considering how closely they were related to the other excerpts under the same codes.

Findings: Interactions and Conflicts Between TCs and State Organizations

In this section, the main characteristics of the interactions between the TCs studied and the government of São Paulo are highlighted, as are the strategies developed by the government to regulate the use of religious activities as part of the treatment proposed by the TCs and the conflicts resulting from this process.

The Incorporation of the Spirituality Category in the Program

The word spirituality is commonly used by all the actors in the field and, most of all, it also appears in the legal documents about TCs and the *Recomeço* Program. For example, the category of spirituality appeared in the 2015 National Drug Policy Council (CONAD) resolution, which regulates TCs' work in the whole country. The document allowed the “development of spirituality” as one of the therapeutic activities provided by these institutions. However, even before the national regulation, the first selection process for organizations participating in the *Recomeço* Program in

São Paulo already allowed for the development of “spirituality activities” without religious discrimination. This means that, since its beginning, the category of spirituality has been incorporated into the norms of the *Recomeço* Program, constituting a category appropriated at the local level.

The inclusion of the category of spirituality into the normative frameworks at national and subnational levels is the result of pressure from TCs and their federations, FEBRACT and others, toward state organizations. It demonstrates that the Brazilian State recognizes and legitimates the religious dimension presented in the treatment model adopted by the TCs and transforms it into a strategy of service implementation, but it guarantees state laicity by substituting the category of spirituality for religiosity.

According to the COED and FEBRACT interviewees, the spirituality category came to replace words related to religious activities (such as “Bible reading, prayers, and worship” for example). Therefore, spirituality appears to circumvent legislation that requires public services to be secular.

I think that who[ever] invented this concept of spirituality was [working for] the State [and trying] to follow the legislation. As the legislation prohibits (religious imposition), then spirituality is allowed. (FEBRACT representative, Interview 64)

This view is also shared by one of the COED managers, who said,

The State is secular, and it won't be able to pay for a religion. So, we use the term spirituality as a subterfuge. We are very clear about this . . . , even if the issue of religiosity and spirituality did not change practices. (COED representative, Interview 62)

The data suggest that interactions between state organizations and TCs resulted in the creation of spirituality as an official category to give a secular character to the treatment developed by these social organizations, which were searching for recognition and financial support from the State. The importance of the category in the service was also captured in the interviews with the TC workers. For them, the question revolved around whether spiritual activities should be mandatory, what types of activities they have to develop, and the role of this category in treatment and, consequently, in reducing drug use.

I see this spiritual part as voluntary because each one has its own faith, their belief, each one was taught in a certain way, and this has to be respected. And, sometimes, certain communities, especially the private ones: “Ah, let's just work on the spiritual.” No, it doesn't have to be forced. But it is always to be brought. Faith is important, but nothing is forced. For example, in the group that exists in my community, it is not mandatory, but if we bring it to the individual because he doesn't want to participate and make him have another look at it and comprehend a different reality, this will make a difference. (Catholic TC, Interview 32)

Although spirituality is not mandatory, according to the TC interviewees, it must be brought into the user's treatment, and it is an issue that has to be worked on with the users, ultimately, "everyone has their own (spirituality)." By including the use of the category spirituality instead of religion in its public selection processes, the state of São Paulo recognized and accepted such practices as part of the service, but adopted a less problematic language that diminished its religious character. It was, therefore, a way to mediate the conflict between state organizations and FBOs.

However, the consequence of the adoption of a concept that mediates the conflict is that both sides have to change. State organizations incorporate ideas and values specific to these TCs, beginning to see like these organizations (Szwako & Lavalle, 2019). TCs also have to adopt new practices and concepts to their services. By accepting the use of spirituality within the TCs, state organizations incorporate the religious culture of these organizations into the policy, recognizing not only the difficulty of prohibiting it as a practice, but also its importance for service users and for TCs. TCs are also affected, as they have to accept this change to keep working for the State. However, the next section shows that the adoption of a mediator concept is not only a matter of language, but also has practical implications for all the FBOs in this research.

State Strategies for Regulating the Use of Religion by TCs

At a first glance, the inclusion of the mediator category of spirituality may reduce the conflicts between the state organization and the TCs. However, part of the conflict is not solved by the new concept, as on one side, COED wants to guarantee that religiosity is diminished and regulated. On the other side, the TCs want to keep doing religious activities and preserving their religious identity. As a consequence, while the state organization accepts the inclusion of spirituality instead of religiosity into treatments, it also tries to regulate it and diminish its centrality in the treatment. As one representative from COED stated, "where religion is strong, there is no technical work and where there is technical work, religion still exists, but it is not as strong" (COED manager, Interview 32).

Analyzing the interviews, we found that COEDs did not intend to prohibit spiritual activities within the TCs (COED manager, Interview 63) as they are allowed in the national and subnational normative frameworks. However, state organizations used different means to regulate and reduce the centrality of these measures in treatment services using four strategies that are not included in the national normative frameworks.

The first is strengthening the presence of specialized professionals in the TCs to "occupy the space that today is occupied by religion" (COED manager, Interview 62). To be part of the *Recomeço* Program, the Government of São Paulo requires FBOs to hire at least one psychologist, a social worker, and a social educator. Representatives of the TCs are also conscious of this professionalization, claiming that it leads to hiring people with more technical and less religious views (Evangelical TCs, Interviews 26 and 32). By creating the idea of professional work versus religious/spiritual treatment, COED has tried to regulate the TCs' service implementation.

The second COED strategy to reduce the space of religiosity and spirituality in the program is the requirement for weekly individual psychological sessions. The goal of the government is to make psychological care as frequent or more frequent than spiritual activities (Bible reading, prayer, worship, and services), also with the expectation of more professional service and the goal to regulate the TCs' activities.

The third strategy is the replacement of workers known as counselors or monitors with social educators. Counselors, common in religious TCs, are typically people who have already been treated in TCs and, after completing their treatment, are hired to work in these organizations. Like the pastors who run the TCs, counselors are also seen as religious figures and play an important role in reproducing religious discourse within the community. Noticing the role of counselors in reinforcing religious culture, COED began to demand their replacement by social educators, people with at least a high-school education undergoing training provided by state organizations. In addition, according to COED interviewees, they maintain a constant dialogue with these actors to identify whether religion is taking precedence over technical knowledge offered by the service (COED manager, Interview 62).

Finally, the fourth strategy developed by COED is requiring alternative activities for users who do not want to participate in the spiritual ones. Some of the TCs develop activities based on the Narcotics Anonymous material, which—although mentioning God—does not involve Bible reading, prayers, and worship. In addition, COED seeks to reinforce the importance of spiritual activities as nonmandatory. The discussion about whether it is mandatory is also felt by the organizations' workers.

This strategy is explained in the following testimonial from a COED manager:

When I took office, I realized that the State had no perception of the religious issue inside the communities. What prevailed was not the technical decision, but what the pastor said, what the priest said or what the counselors said So, our strategy to face this was to demand that all users should undergo to individual care with a psychologist, at least once a week, and with a social worker I needed to empower the professionals I learned something: you never take something out if you don't put something in place. So, what's the use of saying that religion is prohibited if I do not have anything to put in its place? . . . Do you see how we are building strategies to face this conflict? (COED manager, Interview 62)

The changes the state organization proposed to professionalize the TCs are perceived by TC workers as reducing the space for religion in the TCs. According to them, their training and professionalization bring science to these organizations. Furthermore, replacing volunteers with hired professionals is also seen as a strategy to reduce religious centrality, which is in line with the international literature showing that religious organizations have a wide network of volunteers from churches and congregations (Leviton et al., 2006). This phenomenon is exemplified by a psychologist who saw differences in TCs over time:

First, (our work) was more focused on belief, everything was grace or a miracle of God. Not today; we work with human beings, to make them . . . understand the reasons for the

barriers . . . which led them to chemical dependency. So, it is a different view coming from psychology. (Catholic TC, Interview 60)

Furthermore, spirituality in most TCs is restricted to Christianity. Therefore, to broaden the view of the TCs in this area, COED representatives said that the state organization started to discuss other dimensions of spirituality with TC managers and frontline staff. This strategy is intended to develop a critical perspective on spirituality that goes beyond a “Christocentric” (term used by the interviewees) approach. This initiative encountered resistance from the TCs. This was evident when an interviewee from COED said that one of the TCs that is part of the *Recomeço* Program began to implement yoga activities as part of the treatment, and it was criticized by its peers for subverting spirituality.

Such resistance is also found in the literature, and it does not occur only in the case of the *Recomeço* Program (Barroso, 2021). In ethnographic research about online meetings carried out by TC federations during the pandemic, Barroso (2021) heard several statements that show this resistance regarding the initiatives developed by the state to reduce the role of spirituality in the treatment proposed by these organizations, such as “what does God mean to you? Would you take spirituality out of TCs to receive public funding?” . . . “We must have courage and not take away our spirituality!” (Barroso, 2021, p. 98).

Although according to COED interviews this resistance is more present in Catholic and Evangelical TCs, spiritual practices are also present in ecumenical organizations. When the state’s attempts to reduce the role of religion in the treatment proposed by the TCs was discussed, one of the interviewees from an ecumenical TC was asked why there were spiritual practices in his organization, since it had no religious bond. He answered that the organization had already tried to use other forms of dialogue and reflection with users, such as reading Foucault for example, but “Jesus speaks better” (Interview 66). By declaring this, he confirmed the centrality of the religious dimension in the service provided by TCs, as well as the difficulties faced by the state organization in reducing the role of religious practices in the treatment, and he also pointed out that it is a demand from service users.

Conflicts Between State Organizations and TCs

The previous section mentioned the resistance of the TCs to the strategies developed by COED to diminish the role of spirituality in the treatment proposed by TCs. In this section, the conflicts between TCs and COED are explored. The TCs, especially those directly linked to religious institutions/denominations, are resistant to COED’s professionalization initiatives, as there is a widespread belief that “science will reduce the space for belief” (COED manager, Interview 62). Furthermore, there is a concern among these FBOs that their relationship with the state organization will destroy their identities, as previously observed by Bielefeld and Cleveland (2013a). According to Rochester et al. (2010), there are religious principles in FBOs that are non-negotiable and that provide a significant barrier to their relationship with state

organizations. In the case of the TCs, identity is directly related to the centrality of religion both as a form of treatment and as a mission of proselytizing among service users.

Religious identity is one of the main sources of conflict between TCs and the state organization, especially among TCs linked to religious institutions. To keep their identities, TCs adopt many strategies, including a low level of interaction with COED and a dependence on a religious network to solve conflicts. They also try to push their own agenda, for example, hiring professionals with religious ties.

Another source of conflict between TCs and state organizations regards the degree of interference that COED imposes over TCs and the way they are managed. Many TCs' managers are priests or pastors, the organizational culture is based on their religious traditions, and they do not want to change their leadership based on the state's requirements. One interviewee exemplified this conflict:

If you asked me what I don't like (in the TCs), it's this culture of the community leaders, who grip too tightly. They grip in the sense of having huge difficulty in dealing with new things, with insecurity, with losing control. (Evangelical TC, Interview 32)

When state organizations impose rules for implementing the services and also strategies for monitoring and evaluating their actions, this creates a feeling of interference over the TCs, creating a conflict with them. Consequently, although TCs seek to occupy places in State institutional spaces where the policy is disputed (Pires, 2018), some of them, especially those linked to religious institutions/denominations, avoid making demands on the program's State representatives. According to our interviewees, Evangelical and Catholic TCs present fewer demands to the government because their professionals have less freedom to take their issues directly to state organizations and need the consent of the pastor or priest.

Religious TCs avoid making demands because they know they will be denied. They never have specific demands . . . ; there are many more demands from those TCs that are non-religious (ecumenical) and that feel much freer to seek out the State. (COED manager, Interview 62)

TCs also avoid having relations with the state as they can alternatively rely on a wider support network of churches, which helps them to solve structural and financial problems. This religious network was also identified by other authors such as Schneider and Wittberg (2011) and Schneider (2012, 2013). TCs prefer to count on the churches, which often play the role of the State when serving the population (Burity, 2007).

These conflicts impose many challenges. For the state organization, they create difficulties in implementing the services and regulating them.

So, I keep occupying these (religious) spaces and it happens like this, with conflicts They (TCs) get scared I don't know if you know, but resistance to the government is

not easy. The resistance is so great that one of the main references among TCs leads a real campaign against us. He says that the government wants to destroy the identity of TCs. (COED manager, Interview 62)

However, these conflicts also directly impact the service implementation, as the TC professionals are in the middle of the conflict and have to deal with both the government requirements and the TCs' expectations. As one interviewee exemplified,

I see the TC . . . as if it was a small country town, where a *coronel* (the head pastor) is in charge and then a lawyer from the big city (the State) arrives with new ideas, with revolutionary ideas and it produces conflict. For this reason, I speak about the importance of the *Recomeço* Program for this community and for me . . . The TC where I work has a culture from the past, of imposing religion. And now there is this healthy conflict that is bringing the scientific approach of the *Recomeço* Program. This is helping a lot, but it causes a lot of conflict. (Evangelical TC, Interview 32)

Mediating Conflicts

As mentioned previously, the relationship between the TCs and the state organization is marked by different types of conflicts that have consequences for all the actors and, most of all, for service implementation. However, the actors are also interested in stabilizing this relationship, as all of them rely on cooperation for the service to be implemented.

Analyzing the data, we found two main forms of stabilizing the conflict, based on the institutional fit. The first one is regarding the mediator category of spirituality, described in a previous section. The original religious terms used by TCs—prayers, Bible-reading sessions, worship, and so on—would make the partnership unfeasible. As a consequence, the actors developed a process of negotiation and stabilization, creating a concept that mediates the conflict and makes it less evident and visible. This enables a process of symbolic institutional fit. However, as also mentioned, the category is not enough to solve the conflicts, also because the TCs and state organizations develop new strategies to guarantee their interests.

Here is where the second form of institutional fit that enables stabilizing the conflict became central: the participation of FEBRACT as a positional mediator between the state and the TCs. As mentioned, FEBRACT is responsible for selecting, monitoring, and overseeing the TCs of the *Recomeço* Program. In this position, FEBRACT is a mediator between state organizations and TCs. While FEBRACT helps the TCs to adapt to the governments' demands for greater professionalization (through courses and training to its associates), it also presents demands and requirements from the TCs to state organizations.

Interviewer: When TCs have demands, which actors do they search to address them?

Interviewee: I think they go to FEBRACT first. FEBRACT is always closer to them. (FEBRACT representative, Interview 64)

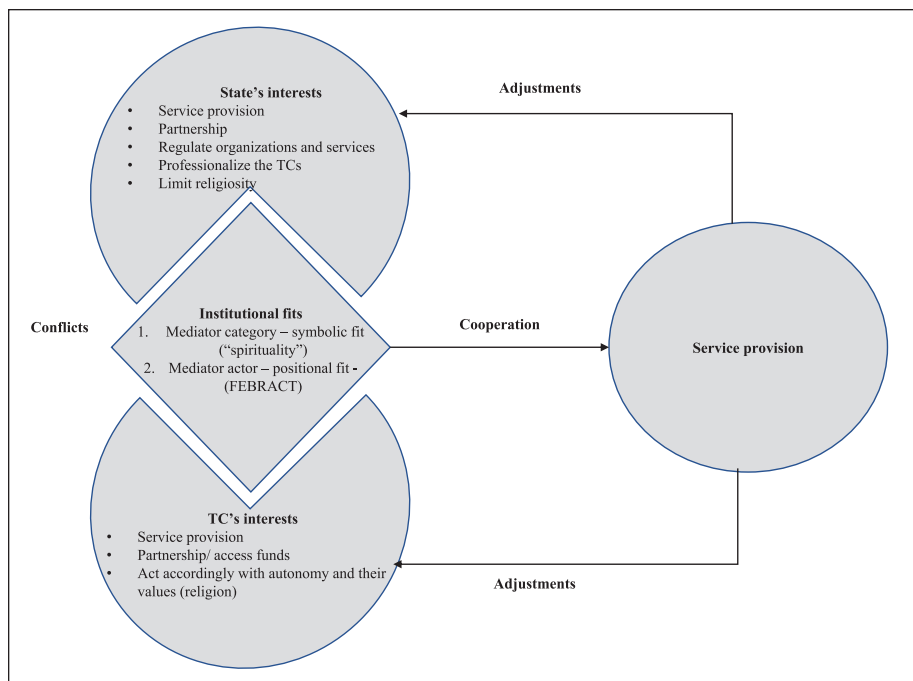


Figure 1. Cooperation and Institutional Fit.

Source. Authors, based on the qualitative data and inspired by Epstein et al. (2015).

Note. TCs = therapeutic communities; FEBRAC = Brazilian Federation of Therapeutic Communities.

This second form of institutional fit is a positional one, in which organizations occupy positions in the relationship with the State. If, on one hand, the two forms of institutional fit generate temporary adjustments, these adjustments do not change the interests and positions of the organizations completely. This is why the stabilization is always temporary and new conflicts may arise in this relationship. Figure 1 summarizes the argument about how cooperation affects the TCs and state organization and is enabled by institutional fit.

Discussion

The article has analyzed how the cooperation between FBOs and state organizations happens, what enables it, and how the cooperation affects both the FBOs and state organizations in a context outside the United States. The findings show how the process of hybridization causes consequences for all the organizations involved and consequences for service implementation. It also shows how the fit is unstable and depends on constant and mutual adjustments.

As the data suggest, state organizations and TCs adopt different strategies to cooperate. These strategies are based on institutional fits that enable adjustments and generate changes in all the organizations and in the service provision. The state organization adjusts the program, considering the changes in the normative framework (such as the inclusion of spirituality), and in the way it interacts with TCs—for example, in the processes of monitoring TCs' activities, in the decisions about the profile of workers. These adjustments directly impact the TCs, which have to change the way they act, adjusting their operations to the state organization requirements. On the other side, the TCs also try to impose changes in the state and service provision, especially negotiating how the normative frameworks will be written and including a mediator—FEBRAC—in this relationship as an actor that mediates the relationship and negotiates the adjustments. The idea of “Jesus speaks better” is central in this argument, as the organizations may try to include the ideas proposed by the state organization, but may end up mixing them with what they think is more effective for the service implementation.

These findings suggest that to stabilize the relationship, the fit is based on forms of mediation. One of them is a symbolic fit (Lavalle & Szwako, 2022). The second one is a positional fit (Abers & de Oliveira, 2015) represented by the inclusion of FEBRAC as the mediator of the relationship between the state organizations and TCs.

The case analyzed here contributes to the literature about hybridization, showing that the cooperation between the actors is conflictual (Beaton et al., 2021), but its stabilization depends on different forms of institutional fits. These fits are based on forms of mediation between the actors, and the capacity of establishing mediation is also important to explain the cooperation. Previous studies also found conflicts around the interactions between state organizations and FBOs (see, for example, Göçmen, 2013). For some of these authors, two variables were central to explain the capacity of FBOs to cooperate with the State: the operational one and the degree of FBOs' religiosity (Fu et al., 2021). Our article shows how—besides these variables—mediation is central to explain cooperation. Actors generate different elements of mediation—which may be symbolic or positional—and these elements create at least temporary stabilization that enables cooperation. This finding contributes to the literature showing how the institutional fit approach may be helpful to understand this type of hybridization between state organizations and FBOs.

The analysis also contributes to the literature about institutional fit, showing that the fit may also create new forms of conflicts, as it generates the need for adjustments to all organizations. This literature defends the mutual constitution between civil society and the State (Abers et al., 2018; Lavalle et al., 2012; Skocpol, 1992). In the case discussed here, while FBOs become more professional, thus reducing the space that was previously entirely occupied by religion, state organizations seek to develop strategies and capacities to confront the centrality of these practices in the treatment offered to the users of the program. This process of cooperation in service implementation brings many conflicts.

These findings may also be explained by the Brazilian institutional context. In Brazil, due to the need to universalize services as proposed by the federal constitution, the state needs to contract organizations to implement public services. In the social assistance area, the FBOs have a long tradition—which began even before the constitution was approved—and the state relies on these organizations to provide several services. On the other side, the FBOs also need the governmental funding to keep providing services. This mutual dependence creates a scenario in which cooperation is less of a desire and more of a need. The consequence is that both sides need to adjust themselves to enable cooperation. Unlike previous studies in the U.S. context (Fu & Cooper, 2020; Fu et al., 2021; Kearns et al., 2005; Shumate et al., 2016, 2018; Sinha, 2012), the case presented here shows how the adjustments are necessary and how the FBOs and state organizations are more likely to change themselves to make the cooperation possible, even if this cooperation is unstable and not ideal for any of the actors. This is a relevant contribution to the hybridization literature in general, and the FBO literature in specific, showing how mutual dependence may affect the way the cooperation works, and the adjustments provoked by it in the hybridization process.

These analyses are limited to the Brazilian context, and future work should explore how much the findings—about mediation, types of institutional fit and mutual dependence—can explain other contexts. In this article, it was not possible to compare how much this process varies considering the type and degree of religiosity between TCs. Future work could also deepen this discussion.

Conclusion

This article has explored what enables the cooperation between FBOs and state organizations in public service implementation in the Brazilian context and what the consequences of this cooperation are for both organizations and the services. For that, the article presented an analysis of the case of the Brazilian *Recomeço* Program, which offers services for drug and alcohol users contracting TCs as organizations that implement the services. The analysis included documents and interviews with different actors involved in the service implementation.

The analysis has contributed to the literature about FBOs, hybridization, and institutional fit, showing how the cooperation between state organizations and FBOs happens, what enables it, and how it affects all the organizations involved in this process. The findings support the initial hypothesis that the State's interactions with social actors such as TCs lead to the incorporation of values and cognitive frameworks by both, generating a mutual constitution between these actors (Abers et al., 2014, 2018; Carlos et al., 2017; Lavalley et al., 2012; Szwako & Lavalley, 2019). It also confirms what the literature on hybridization has identified regarding organizations genuinely cooperating across different institutional environments (Kinney, 2006; Scheirer, 1994; Seibel, 2015). However, the article makes original contributions by showing how different types of institutional fit enable cooperation and affect both FBOs and state organizations, especially when introducing mediators into the relationship.

Appendix

Date	Document	Description
National regulations analyzed		
2001	Resolution 101/2001— Anvisa	Regulates the health licensing of care services to users of psychoactive substances, public and private, especially TCs.
2006	Law No. 11,343/2006	Establishes the National Policy System public on Drugs (SISNAD)
2006	Decree No. 5,912/2006	Regulates Law No. 11,343/2006
2010	Decree No. 7,179/2010	Establishes the “Integrated Plan to Combat Crack
2010	Bill 7.663/2010	Include TCs in the National Drug Law
2011	Letter from Piauí	Defines demands, strategies, and common agreements of TC federations at the national level.
2011	Ordinance No. 3088/2011 of the Ministry of Health	Establishes the Psychosocial Care Network (RAPS)
2011	Resolution 29/2011— Anvisa	Establishes requirements on human resources and infrastructure, as well as criteria related to the use of procedures during treatment and monitoring requirements for TCs
2013	“Crack is Possible to Win” Program	Expands the Integrated Plan to Combat Crack
2013	Law 12,868/2013	Provides for the inclusion of TCs in accessing CEBAS certification
2015	Resolution No. 1— CONAD	Determines that the TCs, despite not being health establishments, can provide voluntary shelter services
2014	PL 8.016/2014	Defends religion as a form of treatment within the TCs
2015	PL 1,377/2015	Proposes legal authorization so that the TCs can talk about religion and develop religious work with the sheltered ones
2016	Ordinance 1482/2016	Creates the Code of the National Registry of Health Establishments (CNES) for the registration of TCs in the municipal health secretariats
2017	Resolution No. 32/2017	Establishes guidelines for strengthening the RAPS, highlighting partnership and intersectoral support for TCs
2017	Interministerial Ordinance No. 2 of the Ministry of Justice and Public Security (MJSP)	Recognizes the work of TCs and allows transfers of public resources

(continued)

Appendix. (continued)

Date	Document	Description
2018	Resolution 01/2018	Gives primacy to abstinence as the main care strategy for users of SPAs by the Brazilian State and the promotion of TCs as part of the social support network for alcohol and other drugs
2018	Ordinance No. 3.449/2018	Establishes the creation of the Therapeutic Communities Committee
2019	Law No. 13,840/2019	Amends the old Drugs Act of 2006 and included a section on welcoming in TCs
2019	Decree No. 9,761/2019	Establishes the National Drug Policy (PNAD), whose abstinence is the main strategy adopted and with specific mentions related to TCs
2019	Ordinance No. 562 of the Ministry of Citizenship	Creates a monitoring plan for Brazilian TCs
2019	Ordinance No. 563 of the Ministry of Citizenship	Creates a register, rules, and procedures for the accreditation of TCs within the scope of the Ministry of Citizenship
2019	Ordinance No. 564 of the Ministry of Citizenship	Creates quality certification for training courses
2019	Technical Note No. 11/2019 of the Ministry of Health	Provides for changes in the National Mental Health Policy and new guidelines for the National Policy on Drugs, with a focus on abstinence.
State regulations analyzed—Recomeço Program		
2013	Decree No. 59,164/2013	Decree creating the Recomeço Program
2017	Work Plan between FEBRACT and COED	Work plan regarding the FEBRACT and COED partnership for the Recomeço Program for the year 2017
2019	Amendment to the Work Plan between FEBRACT and COED	Work plan regarding the FEBRACT and COED partnership for the Recomeço Program for the year 2017
2020	Installation manual for TCs in São Paulo	Manual for the installation and operation of therapeutic communities in the state of São Paulo built by the State Council for Drug Policy
2019	Reference framework for care and intervention in host services offered in TCs in São Paulo	Technical description of care and intervention in reception services offered in TCs of the Recomeço Program—state of São Paulo

Note. TCs = therapeutic communities; CONAD = National Drug Policy Council; FEBRACT = Brazilian Federation of Therapeutic Communities; COED = Drug Policy Coordination.

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